

Prime Medicine

NASDAQ : PRME · \$613M mkt cap · 181M sh · \$149M cash, zero debt · ~\$149M cash = ~9-month runway; FY2025 10-K flags GOING-CONCERN doubt; Young-Company \$200M ATM; Pre-product prime editing (search-and-replace; David Liu / Broad lineage); burn ~\$43M/qtr; PM359 first clinical prime edit (NEJM Dec'25, FDA RMAT Jun'26); May'25 pivot → in-vivo liver (Wilson/AATD)

\$3.39

THE CENTRAL QUESTION

PRME proved the first-in-human prime edit, but holds only ~9 months of cash against a going-concern flag and a \$200M ATM — does the platform tail survive the dilution before it pays off?

Prime Medicine is a clinical-stage prime-editing biotech — the precise 'search-and-replace' gene edit, David Liu's sister tech to BEAM's base editing. Pre-revenue at \$3.12 (~\$564M cap), it proved the first-ever clinical prime edit (PM359, NEJM Dec 2025). Cash is ~\$149M (~9 months), the FY2025 10-K flags going-concern doubt, and a \$200M ATM means severe dilution at a ~\$3 stock; a May 2025 restructuring refocused it on in-vivo liver (Wilson's, AATD). Since then authorizations have accrued — first in-vivo CTA, AATD arbitration win, PM359 RMAT — but not efficacy data, and the financing constraint is unchanged, so the scenario set holds. The DCF asks whether the platform tail can outrun forced dilution. Weighted fair value \$3.39 vs \$3.12 — roughly fair.

PROBABILITY-WEIGHTED EXPECTED VALUE

\$3.39

expected fair value today
+0.0% vs spot \$3.39

FORWARD COMPOUNDED VALUE

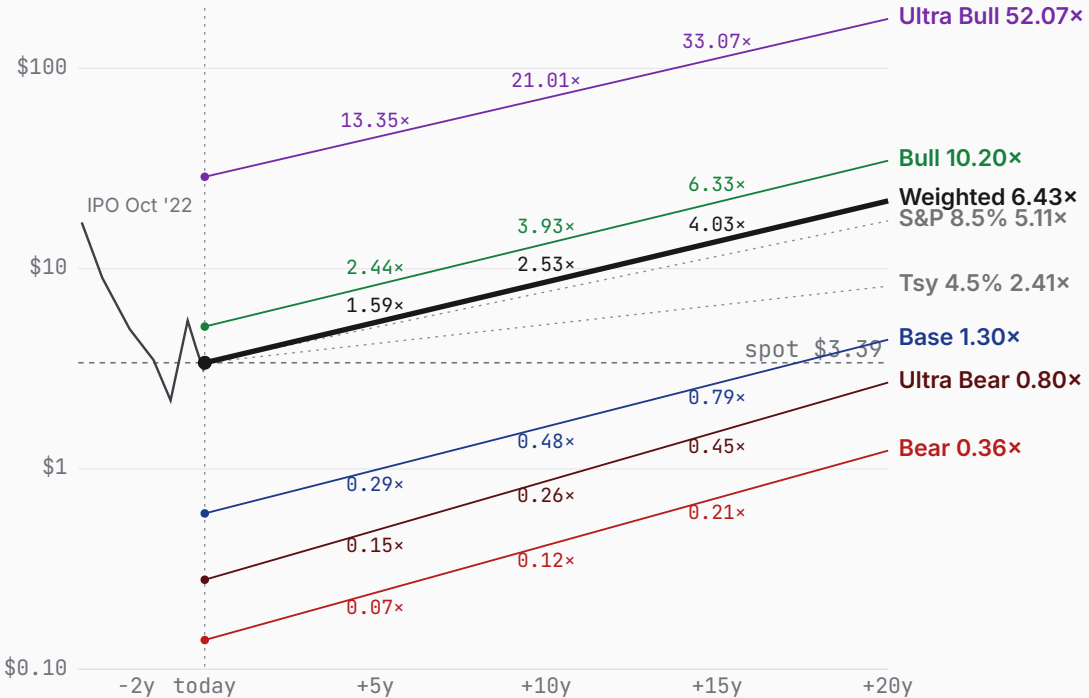
+5y	+10y	+15y	+20y
\$5.39	\$8.58	\$13.67	\$21.78
1.59× spot	2.53× spot	4.03× spot	6.43× spot

WEIGHTING RATIONALE

- Ultra Bear 16%** Programs fail or cash runs out; ATM dilutes to the \$0.40 floor.
- Bear 30%** One program limps; repeated raises at low prices overwhelm holders.
- Base 30%** A liver program clears + BMS pays, but ~\$0.9B of raises nearly triples shares.
- Bull 16%** Multiple liver successes; milder dilution as proof lifts the financing price.
- Ultra Bull 8%** Prime editing wins broadly in-vivo; cleanest share count, platform terminal.

Bull (16%) + Ultra Bull (8%) together contribute \$3.12 — 92% of the \$3.39 expected value despite 24% combined probability. Today's spot (\$3.39) sits 0% below the weighted expected; forward-weighted value crosses spot between +5y and +10y.

PRICE + PROJECTIONS · HISTORICAL THROUGH PROBABILITY-WEIGHTED FORWARD



ULTRA BEAR	\$0.28	BEAR	\$0.14	BASE	\$0.60	BULL	\$5.14	ULTRA BULL	\$28.74
	-91.7% vs spot		-95.9% vs spot		-82.3% vs spot		+51.6% vs spot		+747.8% vs spot
TAM 0.4% P(fail) 90% S2C 1.0 Prob 16%		TAM 1.0% P(fail) 70% S2C 1.1 Prob 30%		TAM 1.7% P(fail) 55% S2C 1.2 Prob 30%		TAM 3.3% P(fail) 30% S2C 1.4 Prob 16%		TAM 6.7% P(fail) 18% S2C 1.6 Prob 8%	
Programs fail; dilutive death-march		One program limps; dilution wins		A liver program ships; dilution offsets		Liver successes; platform validates		Prime editing wins broadly in-vivo	

PROBABILITY WEIGHTS

Ultra Bear 16% Bear 30% Base 30% Bull 16% Ultra Bull 8% · Weighted expected \$3.39 (+0.0% vs spot)

ULTRA BEAR	\$0.28	BEAR	\$0.14	BASE	\$0.60	BULL	\$5.14	ULTRA BULL	\$28.74
	-91.7% vs spot		-95.9% vs spot		-82.3% vs spot		+51.6% vs spot		+747.8% vs spot
Probability 16%		Probability 30%		Probability 30%		Probability 16%		Probability 8%	
Programs fail; dilutive death-march		One program limps; dilution wins		A liver program ships; dilution offsets		Liver successes; platform validates		Prime editing wins broadly in-vivo	
WHY 16%		WHY 30%		WHY 30%		WHY 16%		WHY 8%	
16% — the combination of 9-month runway, going-concern doubt, and clinical-stage risk makes near-term insolvency or a dilutive collapse a genuine, not tail, outcome.		30% — the most likely single path: the science shows enough to keep going but not enough to raise cheaply, so dilution erodes the per-share claim faster than value compounds.		30% — co-equal modal case: clinical proof exists (PM359) and the liver targets are credible, but the model auto-sizes the raises needed to fund them, and that dilution is the binding constraint on per-share value.		16% — requires both clinical breadth across liver targets and a financing path that improves as proof accumulates; plausible given the NEJM precedent, but it needs the science to de-risk before the cash forces bad raises.		8% — the platform-validation tail: requires prime editing to beat base editing and nuclease approaches on versatility and safety at scale, and the equity to survive intact long enough to capture it.	
WHAT HAPPENS		WHAT HAPPENS		WHAT HAPPENS		WHAT HAPPENS		WHAT HAPPENS	
Clinical programs miss and the cash runs out before any partner steps in. With ~9 months of runway and a going-concern flag, the \$200M ATM is tapped repeatedly into a falling stock, sinking the equity toward the \$0.40 distress floor.		A single program advances but slowly, forcing repeated capital raises through the ATM at depressed prices. The platform partly works, yet the financing dominates the equity story — every milestone is paid for in shares.		A Wilson's or AATD in-vivo program reaches market and BMS milestones (>\$3.5B potential) start landing, validating prime editing as a real modality. The business survives and generates revenue at ~37% drug margins.		Several in-vivo liver programs succeed and the platform partly validates as a versatile precise editor, drawing partners and pricing power. Revenue scales toward ~\$1.00B at 40% margins, and earlier validation means raises come at higher prices.		Prime editing establishes itself as a broad in-vivo platform across many edit types, the durable advantage David Liu's chemistry promises. Revenue reaches ~\$2.00B at 45% margins, and validation lets PRME fund growth without punishing dilution.	
FY35 product revenue stalls at ~\$0.11B and p_fail is 90% — survival itself is the bet, not just the science. The share count balloons from 180M to ~702M, so even residual value is spread thin. DCF ~\$0.81; the distress-weighted expected value is ~\$0.28.		FY35 revenue reaches ~\$0.31B with p_fail still 70%. The share count rises to ~568M, diluting surviving holders before economics arrive. DCF ~\$0.46; expected value ~\$0.14 — value is created, but new shares capture most of it.		But getting there costs ~\$0.9B of dilutive raises at depressed prices, nearly tripling the share count to ~500M. FY35 revenue ~\$0.50B; p_fail 55%. DCF \$0.84, expected ~\$0.60 — per-share value lands just below spot because dilution claims the upside.		Because the equity is less distressed, dilution is milder — the share count rises only to ~372M, not the 500M+ of the base. p_fail falls to 30%. DCF \$7.17, expected ~\$5.14 — the first scenario where the platform clearly outruns the balance sheet.		The share count rises only to ~285M — the cleanest balance-sheet outcome — and p_fail drops to 18%. DCF \$34.96, expected ~\$28.74. This is the platform-wins tail that the distressed entry price makes cheap to own.	

Prime Medicine 7 Powers · the moat, scored

Bear \$0.14 Base \$0.60 Bull \$5.14

Arena. In-vivo and ex-vivo precise gene editing for genetic disease, against base editors (BEAM), nuclease editors (Intellia, CRISPR Therapeutics) and editing platforms (Editas).

POWER ORIENTATION
origination window: closing

POWER ORIENTATION · 7 POWERS

Scale Economies	Sub-scale and cash-starved; forced to cut ~25% of staff and the pipeline in May 2025.
Network Economies	No network effects in a therapeutics platform; value is clinical proof, not adoption density.
Counter-Positioning	Prime editing avoids double-strand breaks and makes edit types base editing cannot — a model rivals can't simply copy.
Switching Costs	Switching costs accrue only post-approval, per indication; none yet with zero approved products.
Branding	No commercial brand; pre-revenue clinical-stage with reputational, not pricing, recognition.
Cornered Resource · thesis leans here	David Liu / Broad prime-editing IP and know-how are a genuine scarce asset — but underfunded to exploit.
Process Power	Editing-design and delivery know-how is real but early and not yet a durable manufacturing advantage.

THE RACE · NAMED RIVALS

BEAM (base editing)	\$1.21B cash, runway to ~2029
~30% terminal share · Same gene-editing family; \$1.21B cash vs PRME's \$149M — far better capitalized. A July 2026 binding arbitration put PM647/AATD within PRME's Field (no breach, no damages owed), clearing an AATD IND/CTA for Q3 2026; but Beam-302, the direct AATD rival, advanced to pivotal development (60mg dose), so Beam stays clinically ahead in AATD.	
Intellia / NTLA	well-funded, partnered
~25% terminal share · In-vivo CRISPR nuclease editing; deeper clinical pipeline and balance sheet.	
CRISPR Therapeutics	product revenue + large cash
~20% terminal share · Approved ex-vivo product (Casgevy) and substantial cash; nuclease leader.	
Editas Medicine	constrained
~5% terminal share · Editing platform peer; also capital-constrained and restructuring.	

LEAD / LAG · FALSIFIERS

Clinical first	PM359 first-ever clinical prime edit (NEJM) vs Base/nuclease peers further in pivotal trials	EVEN
Balance sheet	~\$149M / ~9 months / going-concern vs BEAM ~\$1.21B / runway to ~2029	LAGGING
In-vivo delivery	First in-vivo prime-editing CTA cleared (NZ, PM577a/Wilson); Phase 1/2 from 2H26 vs NTLA/peers with in-vivo human data	LAGGING

COMPETITIVE READ

PRME is trying to originate a cornered-resource/process Power in versatile, precise editing — the David Liu prime-editing platform is a genuine scarce asset, and PM359 proves the chemistry works in humans. But the company is cash-starved and was forced to cut its pipeline, so the Power-origination window is closing faster than the science is maturing. The falsifier the bull must clear is concrete — reach a partnered or approved program before the balance sheet forces value-destroying dilution. A July 2026 arbitration win (PM647/AATD within PRME's Field) and the cleared in-vivo CTA move the pipeline toward that bar, but neither is a partnership or an approval, so the window stays closing. Same family as BEAM, opposite balance sheet.



Prime Medicine People · Opportunity · Context · Deal

Bear \$0.14 Base \$0.60 Bull \$5.14

Underwrite the position the way a venture investor underwrites a round — across all four legs, public or private. **People** is the leg scored here (observable only); Opportunity, Context and Deal cross-reference the rest of the memo.

PEOPLE · WHO RUNS / OWNS / FUNDS IT

Allan Reine, M.D. 1 yrs in seat

15.61% HIGH
insider ownership key-person risk

Capital allocation (realized)
Allan Reine became CEO in May 2025 (prior CFO, Jan 2024-May 2025), replacing scientific-founder CEO Keith Gottesdiener in a restructuring that shelved the lead clinical program and refocused the pipeline on liver targets. The company is cash-starved — a going-concern flag and a ~\$200M ATM facility — so capital allocation is dominated by survival and dilution management. Pre-revenue; no dividend or buyback.

Incentive alignment
Directors and officers as a group hold ~15.6% (11 persons, Feb 2026). The clearest positive alignment signal is scientific co-founder David Liu — filed as a director and 10% owner, ~11.2% (~20.2M shares), the largest individual holder — making open-market purchases in June 2025. The new CEO's pay is option-weighted (a 1.3M-share grant in February 2026).

GOVERNANCE FLAGS

- single class of common stock, one vote per share — no dual-class (Delaware)
- classified / staggered board (three classes) — a director-entrenchment flag
- going-concern condition with a ~\$200M ATM — forced-dilution overhang is the central capital-allocation risk
- co-founder David Liu filed as director / 10% owner (also the operating SAB chair) and made open-market purchases June 2025
- ARCH Venture (~10.2%) and GV / Google Ventures (~9.2%) are the largest outside holders

PEOPLE READ

A clean single-class one-vote register and a genuine founder-alignment signal (co-founder David Liu, the largest holder, buying in the open market) are the positives. Against them sit a classified/staggered board, a recent CEO change-plus-restructuring, and — most of all — a going-concern balance sheet with a ~\$200M ATM, so capital allocation is hostage to dilution. Key-person risk is high: the prime-editing platform is founder-science led and the clinical pipeline was just cut to one focus.

THE FOUR LEGS

People
observable composite · 1–5

Opportunity
The scenario distribution · the business snapshot

Context
The 7 Powers analysis (Power Origination)

Deal
Open-market common — entry price weighed against the probability-weighted fair value (the roughly-fair +8.6% finding) + position sizing.



Prime Medicine supporting analysis · disclaimers · glossary

Bear \$0.14 Base \$0.60 Bull \$5.14

PUSHBACK · WHY THE BASE CASE IS TOO HARSH

- 1 **First-in-class proof is real**

PM359 restored NADPH-oxidase activity — the first-ever clinical prime edit (NEJM Dec 2025); FDA RMAT (June 2026) adds a non-dilutive path.
- 2 **Liver refocus de-risks delivery**

The May 2025 pivot to in-vivo liver (Wilson's, AATD) targets the best-validated delivery route, where LNP-to-liver is a solved problem for rivals.
- 3 **Non-dilutive capital exists**

BMS (\$55M upfront, \$55M equity, >\$3.5B milestones) and CFF (up to \$39M) offset some burn — not every dollar of progress must come from the ATM.
- 4 **The de-rating prices the distress**

At \$3.12, down from a \$6.94 high, much of the going-concern risk is already in the stock; the platform tail is being bought cheaply.
- 5 **David Liu lineage**

The Broad-Institute prime-editing IP and scientific lead are a genuine cornered resource that a partner or acquirer could fund through the cash crunch.

FALSIFICATION TRIGGERS

Bull validation PM577a Phase 1/2 (2H26) and a PM647 IND/CTA (Q3 2026) advance toward 2027 data — but the binding trigger is a financing or partnership that closes the runway gap first.	Bear validation Repeated ATM draws at sub-\$3 prices, a clinical setback in the lead liver programs, or runway slipping below two quarters without a partner.	Reframe needed Insolvency, a recap, or a take-under would move this from 'distressed option' to 'broken' — re-underwrite from the floor, not the platform tail.
--	---	---

DISCLAIMERS · PLEASE READ BEFORE USING THIS DOCUMENT

Not investment advice. This document is produced for entertainment and educational purposes only by an individual amateur investor. "Alameda Research 2: Electric Boogaloo" is the unincorporated personal concept of one person and is not a registered investment management entity. Nothing herein constitutes investment advice, a recommendation, or a solicitation to buy or sell any security.	Not a professional. The author is not a registered investment advisor, broker-dealer, CFA charterholder, certified financial planner, or licensed financial professional of any kind. The author has no formal training in equity research or capital markets. No fiduciary relationship is created by reading this document.	AI-assisted analysis. Substantial portions of this analysis were produced with the assistance of large language models. LLMs are known to fabricate facts, miscalculate numbers, hallucinate sources, and present incorrect information with high apparent confidence. Every figure, claim, and projection in this document should be independently verified before acting on it.
Forward-looking statements. Scenarios, valuations, projections, and any forward-looking statements involve substantial assumptions and uncertainty. Actual results may differ materially from those projected. Past performance is not indicative of future results. The model is a model; reality is not.	Author may hold positions. The author may own, intend to acquire, or hold short exposure to \$PRME or related securities at any time. Positions may change without notice and without an update to this document. Do not assume the author's positions match the tone of the analysis.	Do your own research. Do not make investment decisions on the basis of this document. Consult a licensed financial professional and read the company's official SEC filings (10-K, 10-Q, 8-K, proxy) before forming any investment view. If you wouldn't trust your retirement to a chatbot, don't trust this either.

GLOSSARY · CONCEPTS REFERENCED IN THE NARRATIVE

prime editing — Precise 'search-and-replace' gene edit that rewrites DNA without a double-strand break; makes many edit types. David Liu / Broad lineage.	PM359 / CGD — PRME's program for chronic granulomatous disease; the first-ever clinical prime edit (NEJM Dec 2025). Partner-seeking; FDA RMAT (June 2026) aids the BLA path.	in-vivo liver editing — Editing genes inside the body in the liver (Wilson's PM577a, AATD PM647). NZ Medsafe cleared the PM577a application June 2026 — PRME's first in-vivo clinical authorization.
going-concern — Auditor/management doubt, flagged in the FY2025 10-K, that the company has enough cash to operate for the next 12 months.	ATM / forced dilution — An 'at-the-market' equity program (\$200M here) that sells new shares into the open market; at a ~\$3 stock it heavily dilutes existing holders.	p_fail — Modeled probability the scenario ends in failure or insolvency before economics arrive — elevated here because surviving the cash crunch is itself uncertain.